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Counseling for unwanted pregnancies

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Introduction

Changes in the Swiss legislation regarding abortion that entered into force 2002 have freed women and doctors from a criminally defined medical evaluation which was often very difficult and sometimes arbitrary.
In accordance with the time limit regulation which has been in effect since then, the decision is exclusively in the hands of the woman until the 12th week of pregnancy has been completed. At the same time, doctors are obliged to hold a consultation with regard to the planned intervention (medical or surgical termination of pregnancy). Even though many women have already made their decision before the medical consultation, some are ambivalent or made their decision under the influence of a third party. Especially in these conflict situations it is important to consider the following aspects and advice during the consultation.

Objectives and framework of the preoperative counseling consultation

- Informed, self-determined and considered decision-making
- Prevention of abortions that affect the future health of the woman concerned
- Identification of patients at risk for a self-damaging termination of pregnancy

Informed, self-determined and considered decision-making

Informed

- Knowledge (awareness) of the difficulties and problems in carrying the pregnancy
- Knowledge about the aids for carrying the pregnancy (resources)
- Knowledge of the possible short and long-term complications of an abortion, on the physical and mental perspective Knowledge about the aids for carrying the pregnancy (resources)

Self-determined

- Is the patient herself making the decision?
- Is another person making the decision?
- Does the decision correspond with the wishes and values of the patient?

Considered

- Has the decision been made in a crisis or in an emotional state of emergency?
- Have the emotional reactions associated with the unwanted pregnancy been recognized and understood, and have the advantages and disadvantages of the various options been weighed up?
- Has the patient had the opportunity to consider her possible reactions to each option?

Prevention of abortions that are harmful to health

Terminations of pregnancy in case of persistent ambivalence, in case of heteronomy, in acute crisis situations, in case of pre-existing mental illness.

Framework of the consultation before an abortion

- The consultation should take place in a calm setting, with the doctor having sufficient time for and undivided attention on the patient. The patient should be treated with respect and acceptance. The patient is the expert in her problems and values.
- The doctor should practice active listening. This means the following: open questions, mirroring, summarizing, emotional response
- The doctor's questions and information should be formulated in simple language, tailored to the patient's level of education.

Summary and algorithm for counseling consultation

The following list of questions can help the counselor orient themselves to how the patient feels about the unwanted pregnancy on one hand and abortion on the other, thus provide assistance in identifying problem cases and high-risk situations, where further counseling and care is indicated.

With regard to her desire for an abortion, the patient is:

- ambivalent --> clarifying counseling consultation
- decided

Does the wish to have an abortion come primarily from the patient, or has her decision been significantly influece by...

- ... her partner or family? ☐ no ☐ yes → Clarification/couple consultation
- ... financial/economic reasons? ☐ no ☐ yes → Include social services
- Do you have the impression that the patient would like a child but does not believe herself capable (for any reason)?

☐ no ☐ yes → Clarification/couple consultation

Figure 1: Clarification (self-determined)

- Can the patient imagine having an abortion?

  ☐ yes ☐ no → Discuss alternatives

- Are there alternatives to abortion for this pregnancy?

  ☐ no ☐ yes → Discuss alternatives

- How does she think she's gonna feel after the termination of her pregnancy?

  Ideas (relief, sadness): ☐ present ☐ not present → Inform

Figure 2: Clarification (considered)

- Persistent ambivalence ☐ no ☐ yes → Include special counseling
- Suspected mental health issues ☐ no ☐ yes → Include special counseling
- Intense fear regarding the procedure ☐ no ☐ yes → Include special counseling
- Insufficient forethought for consequences ☐ no ☐ yes → Include special counseling
- Foreseeable copingdifficulties ☐ no ☐ yes

Figure 3: High-risk/problematic situations

- Information on methods of abortion (medical or surgical depending on gestational age) and the respective side effects and complications have been provided:

  ☐ yes ☐ no

  ↓

  Referral for abortion

Figure 4: Informed

The phases of the consultation

Introduction and first contact

In the first phase, the patient should have the opportunity to establish an open and trusting relationship with the counsellor. She should feel understood and accepted and be encouraged to speak openly about her problems. The affected woman should have the opportunity to present her conflict with the existing pregnancy.

Important

- Questions those are as open as possible so that the patient can spontaneously present her individual pregnancy conflict. Active listening.

Typical interventions

- “You have come here because you are pregnant, and this pregnancy is problematic. Could you tell me more?”

Main content of the consultation in this phase:

- When and where was the pregnancy established?
- How did the pregnancy occur?
- What were the patient’s reactions?
- Have preliminary talks taken place? With whom, what were the results?
- Does the patient have specific questions and/or expectations for this consultation?

What is the patient’s current emotional state and her current behavior?

- Tension, pressure, panic, fear, etc.

Important

- Recognition of patients with reactions of fear and panic.
- Recognition of patients with emotional ambivalence.

Recording and clarifying the significance of the pregnancy for the patient and her view of the problems in carrying it out

Important

- The experience of pregnancy is the result of the interaction of the individual life situation (external reality) and personal feelings, thoughts and evaluations (internal frame of reference). Both areas (external reality and internal frame of

reference and evaluation) are not static, objective facts, but subjective, potentially changing factors that are subject to numerous influences: Current emotional state, reactions of the environment, previous experiences etc. The pregnant woman must be able to develop a coherent and appropriate picture of the pregnancy and the accompanying problems, on which she can base decisions that are right for her in the medium and long term and that suit her.
- A pregnancy is always an ambivalent experience. It brings together both negative and positive feelings and attitudes (fear of limitations, failure, excessive demands on the one hand and hope for a future with a child, for expansion, growth, love and care, family etc. on the other hand). Pregnant women must be able to perceive and resolve this ambivalence.
- There are often various options for solving the issues associated with the pregnancy. In case of economic problems, evaluation of potential assistance, in case of relationship crises, possibility of couples counseling, etc. These possible solutions may not be taken into account or ignored.

Role of the counselor as a dialog partner for clarifying how the pregnancy is experienced

Typical interventions and questions

- “You've come here because you want to terminate this pregnancy. In your opinion, what problems and difficulties are associated with this pregnancy?”
- “What was your reaction when you realized you were pregnant?”

Comment: Difficulties and problems, just like opportunities and positives, are the result of objective facts and subjective evaluations and assessments.
The counselor should be aware of whether and to what extent she / he can relate to these evaluations.

There can be a clear discrepancy between the evaluation of the patient and the evaluation of the counselor for many reasons:

A) The woman attending counseling may, in the opinion of the counselor, have an unrealistic, distorted view of the burdens of a pregnancy and/or her own resources/ possible support

- “A second child would torpedo all our life plans and disrupt our family.”
- “I won’t be able to finish my education and my career will be completely blocked.”

B) The women attending counseling has a completely different internal evaluation system to the counselor

- “I do not want to take on burdens and restrictions.”
- “We still have debts to pay off right now; maybe in a year I would see it differently."
- “I see nothing positive at all in this pregnancy. It is nothing but horror.”

C) A special case is the patient who cannot and/or does not want to report burdens and difficulties. The reference point or the inner evaluation system is the patient's own unwillingness.

- “I feel that I just don't want. That’s it.”

The following guidelines apply to the counselor:
A) The patient’s internal evaluation and reference system has priority and must be respected.

B) If the counselor has doubts as to the realistic evaluation of the burden, this should be questioned by the counselor and discussed if necessary.

- “Let’s talk again about how professional training would look if you continued the pregnancy.”

C) If the counselor thinks that the patient has a changing, unstable evaluation system with the danger of a significant revaluation after termination of pregnancy, this should be addressed respectfully.

- “There are situations in our lives where we can’t see a way out at first, but later things seem different to how we first thought.”
- “At the moment, you’re under pressure financially and from other burdens. It’s understandable that you feel there is no place for this pregnancy. Do you think that this will be the case after the abortion, or do you think it is possible that you will see or evaluate things differently?”

D) If the counsellor has the impression that the patient is repressing her reference and evaluation system (I don't want to because I don't want to) with the danger that repressed elements of the decision-making process become visible after the abortion and the original subjective evaluations are reversed, this difficult point should be respectfully addressed in order to enable the patient to reduce her resistance.

- "I sense that it is very difficult for you to talk about the stresses (the reasons for

abortion). I also respect your wish not to make any further explanations to a stranger like me, because pregnancy is an intimate area of life that one also wants to protect. My concern, however, is that this "I don't want the pregnancy" includes factors and motives that remain unconscious because they may be painful, unpleasant or embarrassing at the moment. Nevertheless, a dialogue on precisely these aspects could be particularly important in order to prevent the decision from appearing in a completely different light later on."

- "I experience you as deeply hurt at the moment and can understand that you don't want to talk about anything further, which might increase the injury. But there is always the danger that you hurt yourself even more, if you don't have the opportunity, in a consultation, to clarify what is going on inside. Even if it hurts temporarily."

Role of the counselor as a dialog partner for clarifying ambivalence

Typical interventions and questions

- "Were your feelings about this pregnancy always the same, or were there variations, a kind of no and yes or yes and no?"
- "Do you also see any opportunities or positive aspects in this pregnancy?"
- "How does the behavior of your partner or the environment affect your feelings about this pregnancy?"

The aforementioned ambivalence is a normal phase in confronting a pregnancy. The counselor can help the patient through active listening to allow these different, potentially contradictory feelings, and find a solution.

A distinction can be made between the following situations:

A distinction can be made between the following situations:

- A) The woman attending counseling can perceive burdens and opportunities. She makes a conscious evaluation for or against the pregnancy. (stage of resolved ambivalence)
- "At first, I was pleased for a moment. Then it became clear that I can't continue this pregnancy because... "

B) The woman attending counseling can perceive burdens and opportunities, but both sides are more or less equal. (stage of persistent ambivalence)

- "On the one hand, I want a child, but on the other I'm scared of the responsibility. I don't know what I should do."

C) The woman attending counseling can only perceive burdens, no opportunities.

- “This pregnancy is a catastrophe for me. I knew straight away that I have to have an abortion.”

There are two possible reasons for this:

- Opportunities are not present or cannot be perceived.
- Opportunities are repressed and must not be taken for internal reasons (the repressed positive).

The following guidelines apply to the counselor:
A) The counselor should accompany the patient in perceiving and solving the ambivalence.

B) The following applies in cases of persistent ambivalence: give time, help verbalize the ambivalence, increase awareness of implications, persistent ambivalence is more of an argument against abortion.

C) If no positive aspects can be perceived, this should primarily be accepted.

D) If the counselor has the impression that the positive sides are being actively suppressed, this should be questioned respectfully.

- “You say that you immediately experienced this pregnancy as a major catastrophe. Was that because you had never really thought about having a child, because starting a family had not yet been up for discussion?”
- “In very general terms, what are your plans and attitude with regard to a child?”
- “What importance does this have in your life?"

Role of the counselor as a dialog partner for clarifying external or self-determination

If the counsellor gains the impression that the partner or the environment is a decisive factor in experiencing pregnancy or the ambivalence towards it, she should try to differentiate between the following situations.

A) The woman to be counseled can clearly recognize the importance of her partner's behavior and classify it for herself

- "I know well that I would keep the child if he wanted to. But I do not want a child without a partner, I do not want to raise a child alone. It hurts me, but there's no other way."

B) The woman attending counseling is under the intangible influence of her partner or those around her, without this being clear to her

- “I'm very disappointed in him. First he wanted the child too. Now suddenly he doesn’t. I don’t know, I somehow depend on him."
- “My husband is against the child. So I am too. If he doesn’t want it, I don’t either.”
- "I would want to, but my husband won't."

The following guidelines apply to the counselor:

- The patient should, if at all possible, make her own decision, based on her feelings. She should be encouraged towards self-determination and responsibility for herself.
- The influence of the partner should be highlighted and worked through. Couples counseling should be offered, and potential relationship issues discussed.
- If there is a clear discrepancy between the patient’s desire to continue the pregnancy and her partner’s wish to terminate, the decision should not be taken or should be postponed (couples counseling).
- If the partner’s influence is unclear or changing, the decision should not be taken or should be postponed (couples counseling).

Role of the counselor as a dialog partner for finding solutions to problems associated with the pregnancy

Typical interventions

- “You’ve just talked about the financial difficulties having a child would place you in. Now, there are various forms of assistance I would like to discuss with you.”

The patient may not have any information or may be under significant pressure, meaning that potential solutions to the problems associated with the pregnancy have not been considered. Here, the counselor has two roles:

Role 1: She/he can suggest considering alternative solutions

- “Maybe it would be a good idea, in all the agitation, to think again about how you could confront the difficulties you’ve described (financial, family etc.).”

Role 2: She/he can act as a source of information and mediator:

- Passing on the cantonal directory of advice centers and assistance.
- Direct mediation via telephone to, for example, specialist psychotherapeutic assistance.

Assessment of the significance of abortion for the patient and joint anticipation of possible consequences

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Important

- Abortion is a help and a way out of a very stressful situation for many women. Like the pregnancy itself, however, the abortion is usually characterized as an ambivalent experience. This ambivalence (relief and grief) is normal, but for some women it is difficult to comprehend and worrying.
- Abortion is ethically valued very differently from one individual to another and the individual perception may change. This can possibly lead to a changing way of dealing with guilt and feelings of guilt.
- The abortion is not banal for most women, but an important event in their biography.
- It is often first seen as a way to return to the status quo of life before the pregnancy (undoing something). However, most women subsequently find this to be a misconception. Life afterwards is not the same as life before - in other words, the abortion must be accepted or acceptable as part of the patient's life

Role of the counselor as a dialog partner in clarifying the significance of the abortion (benefits and risks) for the patient

Typical interventions:

- "Because of the problems described, you have decided that you do not wish to continue with this pregnancy, and instead have an abortion. So let's talk about abortion."

Dialog means that a reciprocal question and answer exchange, in both directions, takes place. This process comprises various steps in which certain issues should be discussed:

Step 1: Attitudes and prior knowledge of abortion

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The patient should be able to present her view of abortion

- “What is your attitude on abortion?”
- “What risks are you aware of?”

A distinction can be made between three groups

- Women with clear, realistic ideas
- Women with unclear or incorrect ideas
- Women who do not yet have any ideas or thoughts.

Incorrect ideas should be corrected. For women who have not yet developed any ideas, special attention should be paid to steps 2 and 3.

Step 2: Providing information on abortion

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The patient should receive all information that is relevant to her in an understandable form.

As a basis, the following information should be given:

- How is a pregnancy terminated (surgically, with drugs)?
- What methods are used for pain relief and avoiding health risks?
- What are the possible early complications, how common are they and what can be done to treat them?
- What do we know about late physical and mental complications and damages? How can these be avoided as much as possible?

Step 3: Processing and evaluating the information on abortion

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The patient should be able to discuss the personal significance of the information she has received with the counselor. It should be possible for her to ask questions.

- “Have you understood everything I’ve told you about abortion?”
- “How do you evaluate this information? What does this mean for you? Does that make you feel safe or does it make you feel insecure? Do you have any fears or doubts?”
- “Do you have any more questions that have not yet been discussed?”

End of the counseling and summary

Important

- The objective of the counseling is to accompany and support the patient in making an informed, considered and self-determined decision as to whether to continue or terminate the pregnancy.
- There are counseling situations where the woman feels she can’t continue the pregnancy, but can’t go through with an abortion either (“I really don’t want a child, but I can’t kill it”). For these women, it is particularly important that further consultations are offered, and the possibility of adoption is discussed.
- Likewise, women with persistent ambivalence or strong heteronomy should receive a new consultation appointment.
- Women for whom economic burdens are in the foreground should also be offered a new opportunity for discussion after a specialist consultation with a social worker.

Counseling Consultation Flow Chart

Table 1

Phase	Introduction and first contact	Recording and clarifying the significance of the pregnancy for the patient - Problems in the carry out of the pregnancy	Establishing the significance of abortion for the patient – Anticipation of potential consequences	End of the counseling and summary
Functions	1. Creation of a trusting relationship 2. Clarification of expectations	1. Understanding the experience of pregnancy 2. Recognizing and dealing with ambivalence 3. Identifying external influences 4. Discussing potential solutions and assistance	1. Establish knowledge and assessment of abortion 2. Discuss risks and benefits for the patient	1. Summarize phases 1–3 2. Formulate decision 3. Assist with implementation

Literature

Nr.	Title	Author	Journal	Volume	Pages
[P01]	Why prevention of repeat abortion is so challenging: psychosocial characteristics of women at risk	Leeners B, Bieli S, Huang D, Tschudin S.	Eur J Contracept Reprod Health Care	22(1)	38-44
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[P03]	Leitfaden für Beratungsgespräche in sexueller und reproduktiver Gesundheit	n.a.	Sexuelle Gesundheit Schweiz		
[P04]	Schwangerschaftskonflikte – Beratung in der Praxis	Koschorke M.	Klett-Cotta		